Check for updates

Received: 1 August 2022 | Revised: 22 December 2023 | Accepted: 3 May 2024

DOI: 10.1111/famp.13012

FAMILY PROCESS

ORIGINAL ARTICLE

Effects of sexual orientation-based prejudice and discrimination in family of origin on depressive symptoms and life satisfaction in a Chinese sample: Reciprocal and authoritarian filial piety as moderators

Fangsong Liu[1,2] | **Eddie S. K. Chong**[3]

[1]School of Psychology, Jiangxi Normal University, Nanchang, China

[2]Center of Mental Health Education and Research, Jiangxi Normal University, Nanchang, China

[3]Department of Social Work and Social Administration, The University of Hong Kong, Pokfulam, Hong Kong

Correspondence
Eddie S. K. Chong, Department of Social Work and Social Administration, The University of Hong Kong, Room 520, The Jockey Club Tower, Pokfulam, Hong Kong.
Email: echongsk@hku.hk

Funding information
Key Research Base of Humanities and Social Sciences in Universities of Jiangxi Province, Grant/Award Number: JD23060; Seed Fund for Basic Research, The University of Hong Kong, Grant/Award Number: 202107185039

Abstract

Experiencing prejudice and discrimination from family has been found to be positively associated with mental health problems among sexual minorities. Emerging evidence also shows the value of contextualizing the internalization of minority stress by considering individual cultural factors, such as filial piety. We examined whether authoritarian filial piety (AFP) and reciprocal filial piety (RFP) moderated the link between distal stressors in one's family and mental health outcomes. A total of 362 (56.9% male; age: $M = 24.55$, SD = 6.60) Chinese lesbian, gay, bisexual, queer/questioning, and other non-heterosexual (LGBQ+) individuals participated in this study. They provided demographic information and completed a battery of measures for AFP and RFP, sexual orientation-based prejudice and discrimination in family of origin (SOPDF), depressive symptoms, and life satisfaction. Structural equation modeling results showed that SOPDF had a positive and negative link with depressive symptoms and life satisfaction, respectively. In addition, we identified AFP and RFP as significant moderators for the association between SOPDF and depressive symptoms, and the association between SOPDF and life satisfaction, respectively. Specifically, the positive effect of SOPDF on depressive symptoms was greater for participants with higher levels of AFP; the negative effect of SOPDF on life satisfaction was greater for participants who endorsed higher levels of

This study was supported by the Key Research Base of Humanities and Social Sciences in Universities of Jiangxi Province (JD23060).

This is an open access article under the terms of the Creative Commons Attribution-NonCommercial License, which permits use, distribution and reproduction in any medium, provided the original work is properly cited and is not used for commercial purposes.
© 2024 The Author(s). *Family Process* published by Wiley Periodicals LLC on behalf of Family Process Institute.

RFP. Our findings corroborated past studies' conclusion about the detrimental impact of familial sexual stigma on LGBQ+ people's mental health. Furthermore, such impact on negative and positive mental health outcomes are respectively conditioned by the degree to which LGBQ+ individuals endorse AFP and RFP. These findings underscore the importance for therapists who endorse family therapy to help LGBQ+ clients navigate familial sexual stigma and consider the role of filial piety beliefs in shaping the impact of familial sexual stigma on these clients' mental health.

KEYWORDS

authoritarian filial piety, depressive symptom, life satisfaction, reciprocal filial piety, sexual orientation-based prejudice and discrimination in family of origin

INTRODUCTION

Sexual stigma experienced in one's own family of origin has important health implications for many individuals who identify as lesbian, gay, bisexual, queer/questioning, and non-heterosexual (LGBQ+). Research has shown that sexual orientation-based prejudice and discrimination from a family of origin (SOPDF) has detrimental effects on the mental health of LGBQ+ youths and young adults across cultures (Liu et al., 2021a; Parra et al., 2018; Puckett et al., 2015; Shao et al., 2018). For instance, SOPDF was positively related to depressed mood and anxiety (Parra et al., 2018; Puckett et al., 2015; Shao et al., 2018) and negatively linked to emotional, social, and psychological well-being (Liu et al., 2021b; Shao et al., 2018). Health disparity research has also shown that LGBQ+ individuals disproportionately reported greater levels of depression and lower levels of life than their heterosexual counterparts and sexual stigma has been proposed as a source that contributes to the observed mental health disparity (Meyer et al., 2022; Rosario et al., 2014).

The prevalence of familial and parental sexual stigma has been documented across regions (Beijing LGBT Center, 2014; Davis & Anderson, 2021). According to a nationwide survey conducted in China (Suen et al., 2022), involving 11,048 lesbian and gay adults, researchers discovered that 54.2% of participants reported encountering negative treatment from their family members due to their sexual orientation and/or gender expression whereas only 6.3% of respondents perceived an accepting attitude from their families. Similarly, a survey of 1197 U.S. nationally representative sexual orientation and gender minority adults found that about four in 10 reported that at some point in their lives they were rejected by a family member or close friend because of their sexual orientation or gender identity (Pew Research Center, 2013). In view of the prevalence of familial sexual stigma and its implication on the mental health of LGBQ+ individuals, it is imperative to identify variables that intensify or soften these links.

Among potential moderating variables, emerging evidence indicates that personal values and beliefs may provide important context for understanding the connections between SOPDF and mental health outcomes (Liu & Chong, 2023). One such variable is filial piety, which refers to a cultural expectation where individuals prioritize their family's honor, foster family harmony, and fulfill familial obligations in return for parental care and support (Bedford & Yeh, 2021). Although filial piety belief serves an important social function of protecting the well-being of both parents and their children given the reciprocal exchange of care, it may also incur stress to both parties when their needs conflict with each other (Bedford & Yeh, 2021).

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/famp.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

In view of the differences in what motivates individuals to practice filial piety, how filial piety plays a role in the link between perceived familial sexual stigma and well-being remains to be answered. This study aimed to address this knowledge gap to generate insights for developing culturally responsive strategies for LGBQ+ individuals to navigate familial sexual stigma.

Sexual orientation-based prejudice and discrimination from family of origin

Despite important legal and diagnostic changes, such as the decriminalization of same-sex sexual acts in 1997 and the removal of same-sex sexual orientation as an illness in the Chinese Classification of Mental Disorders in 2001 (Mann, 2011), non-heterosexual orientations continue to face stigma and social challenges in Mainland China. For example, tolerance toward same-sex sexual act was low among the general population (Tang & Pan, 2018); nearly 70% of citizens in urban areas of China regarded same-sex sexual orientation to be morally wrong (Li & Zheng, 2013). Furthermore, sexual orientation-based prejudice and discrimination is most frequently occurred in family setting (Beijing LGBT Center, 2014; Suen et al., 2022). In a heteronormative society, family members of LGBQ+ individuals often have deep-rooted ideas that heterosexuality is the only normal and legalized sexual identity, which may lead their family members to express anti-LGBQ+ attitudes and behaviors regardless of the outness of the person (DeChants et al., 2022). In the Chinese society where family reputation and "face" are placed as a priority, having a non-heterosexual member within a family is considered as a disgrace (Kam, 2007). When parents of non-heterosexual children speculate or know their children' sexual orientation, they may not only feel ashamed that they did not raise a "good" child, but also fear that their family could become a target of gossip and lose face (Choi & Luo, 2016). Such negative thoughts and feelings may cause family members to demonstrate negative attitudes toward non-heterosexual orientation, perform emotional rejection and physical abuse toward LGBQ+ members (Friedman et al., 2011), or force LGBQ+ members to receive conversion therapy to change their sexual orientation (Chan et al., 2022).

According to the minority stress theory (Meyer, 2003), external sexual prejudice and discrimination from the family of origin have a detrimental impact on psychological health. Extant evidence suggested that sexual orientation-based rejection from parents and other family members was positively associated with mental health problems, such as depression, anxiety, and suicidality (Parra et al., 2018; Puckett et al., 2015; Willoughby et al., 2010). However, research on the relationship between SOPDF and positive mental health was relatively rare. Based on the dual-continuum model of mental health (Keyes, 2005), mental illness and mental well-being are two divergent but related psychological constructs. This model suggests that the absence of psychopathologies (e.g., depression and anxiety) does not necessarily represent mental health. Compared with mental well-being, previous studies among sexual minorities primarily focused on psychological problems (Bariola et al., 2017). To comprehensively understand the contribution of SOPDF to mental health among Chinese sexual minorities, it is necessary to examine the effect of SOPDF on negative and positive mental health.

Depression is a leading cause of disability worldwide (World Health Organization, 2017), and research has shown that rates of depression are significantly higher among LGBQ+ individuals compared to their heterosexual counterparts (Rosario et al., 2014). In addition, life satisfaction, which represents the cognitive aspect of subjective well-being, can be seen as a holistic gauge of how well an individual's aspirations align with their achievements (Cummins, 2013). Its importance as an indicator of overall well-being for the entire population is emphasized by its use by the World Health Organization (WHO; Lindert et al., 2015). Previous research has indicated that sexual minority individuals generally experience lower levels of life satisfaction than their heterosexual counterparts (Meyer et al., 2022). Therefore, this study selected depression and life satisfaction as indicators of negative and positive mental health, respectively.

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/famp.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

Filial piety

The dual filial piety model (DFPM; Yeh & Bedford, 2003) posits that filial piety consists of two fundamental aspects: Reciprocal filial piety (RFP) and authoritarian filial piety (AFP). RFP refers to individuals' genuine gratitude for their parents' efforts, sacrifice, and rearing, and displays itself through the voluntary support and care for their parents and ancestors (Bedford & Yeh, 2019). The development of RFP stems from an individual's need for interpersonal affection and mutual relatedness. The expression of RFP is motivated by equal, reciprocally respectful, and intimate interactions between parent and child. In contrast, AFP manifests itself through obedience to and gratification of parental demands out of social expectations prescribed to the role of a child (Bedford & Yeh, 2019). The development of AFP originates from an individual's need to belong to a larger social group (e.g., family), where membership is deemed worthy by deference to authority (i.e., parents) and suppression of autonomous expression. In sum, RFP and AFP reflect divergent motivating factors that shape the ways in which individuals negotiate between their own needs and familial interests (i.e., mutual relatedness vs. social belongingness).

Given that RFP and AFP influence how individuals approach boundary negotiation, these individual cultural factors may change how anti-LGBQ+ treatment from family members impact LGBQ+ individuals' mental health. Specifically, for LGBQ+ individuals with high levels of AFP, they get accustomed to prioritize parental needs and familial/social rules at the expense of authentic self-expression (Bedford & Yeh, 2019). When they encounter anti-LGBQ+ messages and treatment from family members, it might be a challenge for them to "push back" lest being disowned or shunned out of the family (Liu et al., 2021a). Over time, such suppression of autonomy may exacerbate the extent to which biased treatment from family contributes to own psychological distress. SOPDF communicates the message that their parents/family members are disappointed about their sexual orientation and they would receive punishment (e.g., emotional neglect, verbal and physical abuse), which may cause them to emotionally feel guilty and behaviorally hide or reject their sexual orientation. In this sense, SOPDF may have a stronger detrimental impact on mental health for Chinese sexual minorities who endorse high levels of AFP.

RFP was positively associated with emotional intelligence (Chen et al., 2018) and higher cognitive flexibility (Jen et al., 2019), suggesting that individuals with higher levels of RFP have more cognitive resources to deal with interpersonal conflicts. On the one hand, researchers found that RFP mitigated the positive association between perceived public stigma and internalized homonegativity, suggesting that sexual minorities who endorse high levels of RFP are more capable of protecting their LGBQ+ related self-concept from heterosexist beliefs and behaviors (Liu et al., 2021b). When family displays rejection and discriminative behaviors, these individuals might be less likely to attribute blame to themselves or develop negative feelings about their sexual orientation. The mental health of individuals with greater RFP may, therefore, be impacted by SOPDF to a smaller degree than those with lower levels of RFP. On the other hand, individuals with high RFP tend to be raised in a family with reciprocal love, respect, and care (Bedford & Yeh, 2019). Anti-LGBQ+ sentiment in the family of origin may cause LGBQ+ individuals with high RFP to question the mutual support and solidarity they have experienced growing up. Given the contrasting functions of RFP, we examined the moderating role of RFP as a research question in the association between SOPDF and mental health.

The present study

Based on previous theories and empirical evidence, we proposed the following hypotheses: (a) SOPDF would be positively and negatively associated with depressive symptoms and life satisfaction, respectively, and (b) high levels of AFP would amplify the detrimental impact of

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/famp.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

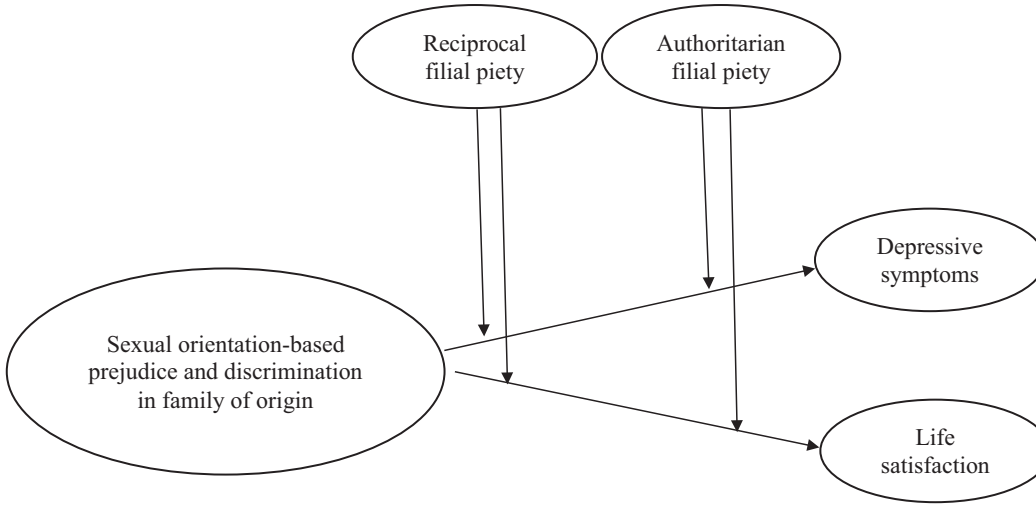

FIGURE 1 Research model.

SOPDF on mental health outcomes. In addition, we raised a research question: What is the moderating role of RFP in the associations of SOPDF with depressive symptoms and life satisfaction? The hypothesized moderation model was illustrated in Figure 1.

METHOD

Participants and procedure

Researchers obtained ethical approval from the research ethics committee of Hong Kong University. Targeted and snowballing sampling was used to recruit participants. Recruitment messages were distributed through the LGBT social media of the Beijing LGBT Center, which was one of the largest LGBT organizations in Mainland China providing non-profit services for sexual minorities. Specifically, a recruitment advertisement that includes research purposes and relevant links for accessing the online survey was posted on the WeChat Official of the Beijing LGBT Center that has been subscribed by many Chinese sexual minorities. The subscribers were encouraged to forward the advertisement to their personal contacts and sexual minority-related WeChat groups. The potential participants who expressed interest could click on the links or scan the QR code to visit the online survey with the consent form on the first page. After reading the consent form which informed participants that their participation was voluntary, their personally identifiable information was anonymized, and they had the right to stop answering survey items at any time without any negative consequences. Participants were required to provide informed consent prior to their commencement of the study. Upon completion of the online survey, the participants could receive 6 RMB ($US 0.94) or donate the fee to LGBQ+ non-profit organizations as compensation for their time and efforts.

The inclusion criteria consisted of people who (a) were at least 18 years old, (b) were cisgender, (c) self-identified as lesbian, gay, bisexual, questioning/queer, and other non-heterosexual individuals (LGBQ+), and (d) resided in Mainland China. Even though transgender and gender nonconforming individuals can also be sexual minorities, this study focused on sexual orientation-salient experiences (i.e., SOPDF) rather than intersectional experiences. To refrain from generating misleading conclusion about individuals who identify as both sexual orientation and gender minorities, transgender and gender nonconforming individuals were not included in this study. Of the

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/famp.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

375 sexual minority individuals who met the inclusion criteria and provided informed consent, 13 participants did not answer correctly at least one of the two validity items (e.g., "please select the second choice"). Finally, 362 (56.9% male; age: $M = 24.55$, SD = 6.60) participants were included in the subsequent analyses. The participants' detailed demographic information is shown in Table 1.

TABLE 1 Participants' demographic information ($N = 362$).

Demographic information	*N* (%)
Gender	
Female	156 (43.1)
Male	206 (56.9)
Sexual orientation	
Gay	167 (46.1)
Lesbian	70 (19.3)
Bisexual	84 (23.2)
Questioning/Queer	16 (4.4)
Pansexual	16 (4.4)
Asexual	9 (2.5)
Age	
18–25 years old	258 (71.3)
26 to 35 years old	76 (21)
More than 35 years old	28 (7.7)
Marriage status	
Unmarried	333 (92)
Married	14 (3.9)
Divorced	7 (1.9)
Others (e.g., fake marriage)	8 (2.2)
Same-sex partner status	
Not having same-sex partner	204 (56.4)
Having a same-sex partner	145 (40.1)
Not sure	13 (3.6)
Place of birth	
Urban area	259 (71.5)
Rural area	103 (28.5)
Educational background	
High school or below	66 (18.2)
Associate degree	40 (11)
Bachelor's degree	196 (54.1)
Master's degree or above	60 (16.6)
Monthly income	
2000 RMB or below	179 (49.4)
2000 to 4999 RMB	78 (21.5)
5000 to 9999 RMB	53 (14.6)
10,000 to 19, 999 RMB	35 (9.7)
20,000 RMB or above	17 (4.7)

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/famp.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

Measures

Sexual orientation-based prejudice and discrimination in family of origin

The three family-related items in the Heterosexist Harassment, Rejection, and Discrimination Scale (HHRDS; Szymanski, 2006) were used to assess the degree to which respondents experienced sexual prejudice and discrimination in their family of origin. The HHRDS consisted of 14 items rated on a 6-point Likert-type scale ranging from 1 (*the event has never happened to you*) to 6 (*the event happened almost all of the time*). These 14 items were previously adopted to measure individuals' sexual orientation-based prejudice and discrimination in different contexts, including family, friends, workplace, school, people in helping jobs, people in service jobs, and strangers. Because the present study focused on sexual orientation-based prejudice and discrimination experiences in families, the three family-related items (i.e., "How many times have you heard anti-lesbian/anti-gay/ anti-bisexual remarks from family members?" "How many times have you been rejected by family members because you are a lesbian/gay/bisexual individual?" and "How many times have you been treated unfairly by your family because you are a lesbian/gay/bisexual individual?") were selected and used for this study. The Chinese version of the HHRDS has been translated into previous research (Liu et al., 2022) and was used in this study. This scale has shown satisfactory reliability and validity in previous research (Szymanski, 2006). For example, the internal reliability scores were 0.90 for lesbians (Szymanski, 2006) and 0.91 and for gay and bisexual individuals (Szymanski, 2009). In addition, the three items of SOPDF were found to have good internal reliability and construct validity among Chinese sexual minorities (Liu et al., 2022). Based on the current sample, the Cronbach α value of the three items in the present study was 0.78. The average score of the three items was used in the correlational analysis, with a higher score indicating a higher level of SOPDF.

Filial piety

The Filial Piety Scale (Yeh & Bedford, 2003), originally developed in Chinese, was adopted to measure the extent to which respondents endorsed RFP and AFP. This scale includes 16 items with two subscales: RFP and AFP. These items were rated by a six-point Likert-type scale (1 = *strongly disagree* to 6 = *strongly agree*). Sample items for the RFP Subscale were "be grateful to parents for raising you" and the AFP Subscale was "live with parents even after marriage." This scale has been used among the general and sexual minority populations, and showed good internal reliability and validity (Liu et al., 2021a, 2021b). In the present study, the Cronbach α values for the RFP and AFP Subscales were 0.89 and 0.86, respectively. The RFP and AFP scores were obtained by averaging the corresponding item scores and higher scores suggested higher levels of RFP and AFP.

Depressive symptoms

The nine-item Patient Health Questionnaire (PHQ-9; Kroenke et al., 2001) was utilized to assess the degree to which LGBQ+ individuals had depressive symptoms. The questionnaire used in the present study was previously translated into Chinese in a prior study (Chen et al., 2010). All items are rated on a 4-point Likert-type scale (0 = *not at all* to 3 = *nearly every day*). This scale has been widely used across cultures and has shown adequate reliability and validity. For example, the internal reliability of the PHQ was 0.86 to 0.89 among U.S. patients (Kroenke et al., 2001) and 0.91 among Chinese patients (Chen et al., 2010). A sample item was "Feeling down, depressed, or hopeless." Based on the current sample, the Cronbach α value of this scale

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/famp.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

in this study was 0.92. The average score of PHQ-9 was used in the correlational analysis, with a higher score suggesting a higher level of depression.

Life satisfaction

The Satisfaction With Life Scale (SWLS; Diener et al., 1985) was used to assess the degree to which respondents were satisfied with their lives. This scale is consisted of five items which are rated on a seven-point Likert-type scale ranging from 1 = *strongly disagree* to 7 = *strongly agree*. Wu and Yao (2006) translated and validated the Chinese version of SWLS, which was used in the present study. The SWLS has been widely used in previous research and recognized as a reliable and valid measure for life satisfaction (Michaels et al., 2019; Wu & Yao, 2006). A sample item for this scale was "I am satisfied with my life." Based on the current sample, the Cronbach α value of this scale in this study was 0.91. The scale score was obtained by averaging all item scores and a higher score suggested a higher level of life satisfaction.

Data analysis

Descriptive analyses were conducted using SPSS 22.0. The skewness and kurtosis values of the key variables were computed to examine whether they had a normal distribution. Based on the absolute-value cutoff score (3.00 and 8.00 for skewness and kurtosis respectively; Kline, 2005), all variables were normally distributed. In addition, no multivariate outliers were detected using the Mahalanobis distance test. The means and standard deviations of all variables were computed to describe the demographic characteristics. In addition, the Pearson correlation analyses were conducted to compute the correlational coefficients among variables. If demographic variables had significant correlations with life satisfaction, they were included as covariates for the outcome variables in the subsequent path analysis.

Structural equation modeling (SEM) analyses were conducted using Mplus 7.4 to test the hypothesized moderation model. A two-step approach was used to perform SEM analyses (Anderson & Gerbing, 1988). First, confirmatory factor analysis (CFA) was conducted to examine the fit of the measurement model with latent variables of SOPDF, RFP, AFP, SOPDF × RFP, SOPDF × AFP, depressive symptoms, and life satisfaction. Then the structural model was tested to examine the moderating roles of AFP and RFP in the associations of SOPDF with depressive symptoms and life satisfaction. The goodness-of-fit of the model was evaluated by using the following fit indices: Tucker-Lewis index (TLI), Comparative Fit Index (CFI), and Root Mean Square Error of Approximation (RMSEA; Hu & Bentler, 1999). The criteria for acceptable model fit include TLI values of 0.90 or above, CFI values of 0.90 or above, and RMSEA values of 0.08 or below (Hu & Bentler, 1999).

RESULTS

Descriptive analyses

The means and standard deviations of all variables, and correlational coefficients between variables are illustrated in Table 2. SOPDF was positively and negatively correlated with depressive symptoms and life satisfaction, respectively. Being older and having a same-sex partner were correlated with higher life satisfaction. These two demographic variables were included as covariates for life satisfaction in the subsequent analyses.

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/famp.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

TABLE 2 Means, standard deviations, and correlational coefficients for variables.

Variables	1	2	3	4	5	6	7	8	9	10	11
1. Age	1										
2. Gender [a]	−0.21**	1									
3. Relationship status [b]	−0.07	−0.04	1								
4. Place at birth [c]	0.09	−0.25**	−0.06	1							
5. Educational level	0.17**	0.06	−0.06	−0.07	1						
6. Monthly salary	0.54**	−0.03	−0.16**	−0.01	0.24**	1					
7. SOPDF	−0.09	0.16**	−0.05	0.01	−0.04	−0.04	1				
8. AFP	0.15**	−0.26**	0.08	0.21**	−0.04	0.01	−0.11*	1			
9. RFP	0.11*	−0.05	0.06	−0.03	0.02	−0.01	−0.05	0.30**	1		
10. Depression	−0.09	0.09	0.08	0.06	−0.08	−0.01	0.21**	0.01	−0.07	1	
11. Life satisfaction	0.14**	0.02	−0.16**	−0.07	0.07	0.16**	−0.27**	0.04	0.10	−0.48**	1
M	24.55	0.43	0.60	0.28	4.70	2.69	2.14	2.14	0.01	0.86	3.92
SD	6.60	0.50	0.49	0.45	1.00	1.58	1.10	0.78	0.771	0.75	1.46

Note: Gender[a] was dummy coded, 0 = male, 1 = female; Relationship status[b] was dummy coded, 0 = having a same-sex partner, 1 = having no same-sex partner or unsure; Place at birth[c] was dummy coded, 0 = urban area, 1 = rural area.

Abbreviations: AFP, authoritarian filial piety; RFP, reciprocal filial piety; SOPDF, sexual orientation-based prejudice and discrimination in family of origin.

$^{*}p < 0.05$, $^{**}p < 0.01$.

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/famp.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

Test of the measurement model

CFA was first conducted to examine the fit of the measurement model. Three family-related items of the HHRDS Scale were used as indicators of SOPDF; nine items of the PHQ were used as indicators of depressive symptoms; and five items of the SWLS were used as indicators of life satisfaction. The item parceling method (Russell et al., 1998) was used to create three parcels for the latent variables of AFP and RFP. To create indicators for the interaction between SOPDF and AFP, as well as between SOPDF and RFP, the scores of three SOPDF items were respectively paired with the scores of three AFP parcels, or RFP parcels (Marsh et al., 2004). CFA results indicated that the measurement model was fit to the data (CFI = 0.942; TLI = 0.933; RMSEA = 0.052, 90% CI [0.047, 0.058]). Factor loadings of all indicators ranged from 0.52 to 0.96 (see Table 3 for more information about factor loadings), suggesting that these latent variables were adequately measured by their corresponding indicators.

TABLE 3 Standardized factor loadings for the measurement model.

Latent variable	Indicator	*B*	SE	*p*
SOPDF	Item 1	0.93	0.03	< 0.001
	Item 2	0.81	0.03	< 0.001
	Item 3	0.52	0.04	< 0.001
RFP	Parcel 1	0.88	0.02	< 0.001
	Parcel 2	0.83	0.02	< 0.001
	Parcel 3	0.88	0.02	< 0.001
AFP	Parcel 1	0.80	0.03	< 0.001
	Parcel 2	0.88	0.02	< 0.001
	Parcel 3	0.80	0.03	< 0.001
SOPDF × RFP	Parcel 1	0.96	0.03	< 0.001
	Parcel 2	0.80	0.03	< 0.001
	Parcel 3	0.53	0.04	< 0.001
SOPDF × AFP	Parcel 1	0.75	0.06	< 0.001
	Parcel 2	0.65	0.05	< 0.001
	Parcel 3	0.54	0.06	< 0.001
Depressive symptoms	Item 1	0.80	0.02	< 0.001
	Item 2	0.86	0.02	< 0.001
	Item 3	0.72	0.03	< 0.001
	Item 4	0.77	0.02	< 0.001
	Item 5	0.69	0.03	< 0.001
	Item 6	0.77	0.02	< 0.001
	Item 7	0.71	0.03	< 0.001
	Item 8	0.73	0.03	< 0.001
	Item 9	0.76	0.03	< 0.001
Life satisfaction	Item 1	0.92	0.01	< 0.001
	Item 2	0.94	0.01	< 0.001
	Item 3	0.88	0.01	< 0.001
	Item 4	0.84	0.02	< 0.001
	Item 5	0.59	0.04	< 0.001

Abbreviations: AFP, authoritarian filial piety; RFP, reciprocal filial piety; SOPDF, sexual orientation-based prejudice and discrimination in family of origin.

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/famp.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

Test of the structural model

We then conducted SEM analysis to examine the hypothesized moderation model with SOPDF as predictor, RFP and AFP as moderators, depressive symptoms and life satisfaction as outcomes, and demographic variables (i.e., age, relationship status, and monthly salary) as covariates. Results indicated that the structural model had satisfactory fit to the data (CFI = 0.941; TLI = 0.934; RMSEA = 0.047, 90% CI [0.042, 0.053]). As illustrated in Figure 2 and Table 4, SOPDF was positively and negatively associated with depressive symptoms ($\beta = 0.26$, $p < 0.001$) and life satisfaction ($\beta = -0.26$, $p < 0.001$), respectively. AFP had a moderating effect on the association between SOPDF and depressive symptoms ($\beta = 0.17$, $p = 0.02$). Moreover, RFP had a moderating effect on the association between SOPDF and life satisfaction ($\beta = -0.13$, $p = 0.02$). The conditional direct effects of SOPDF on depressive symptoms and life satisfaction was further examined when AFP and RFP were at five different levels: Very high (two SDs above mean value), high (one SD above mean value), average (mean value), low (one SD below mean value), and very low levels (two SDs below mean value). As shown in Table 5, the positive effect of SOPDF on depressive symptoms was stronger for participants who endorsed very high levels of AFP ($B = 0.53$, $p = 0.002$) than those with very low levels of AFP ($B = -0.17$, $p = 0.234$); the negative effect of SOPDF on life satisfaction was stronger for participants with very high levels of RFP ($B = -0.66$, $p < 0.001$) than those with very low levels of RFP ($B = -0.03$, $p = 0.87$). This model accounted for 7.5% and 15.2% of the variance in depressive symptoms and life satisfaction, respectively.

DISCUSSION

This is the first study to examine the moderating roles of AFP and RFP in the associations of SOPDF with depressive symptoms and life satisfaction. As expected, SOPDF was positively and negatively associated with depressive symptoms and life satisfaction, respectively. In addition, the positive effect of SOPDF on depressive symptoms was stronger when participants

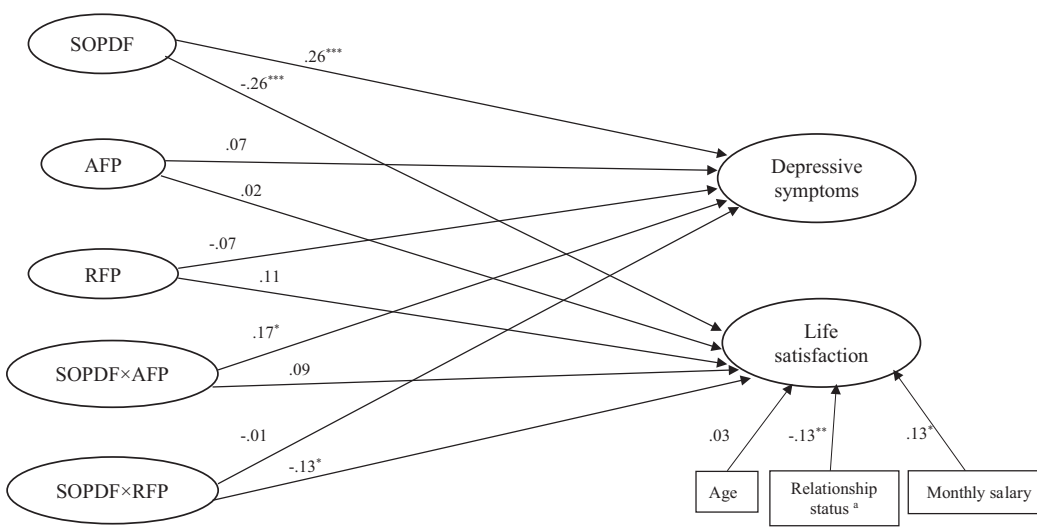

FIGURE 2 Moderation model of authoritarian and reciprocal filial piety, sexual orientation-based prejudice and discrimination in family of origin, and mental health outcomes. *Note*: All coefficients are standardized. *$p < 0.05$, **$p < 0.01$, ***$p < 0.001$. SOPDF=Sexual orientation-based prejudice and discrimination in family of origin. AFP, authoritarian filial piety; RFP, reciprocal filial piety. Relationship status [a] was dummy coded, 0 = having a same-sex partner, 1 = having no same-sex partner or unsure.

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/fam.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

TABLE 4 Results for the hypothesized moderation model.

Variables	Depressive symptoms		Life satisfaction	
	β (*SE*)	*p*	*β* (*SE*)	*p*
SOPDF	0.26 (0.06)	<0.001	−0.26 (0.06)	<0.001
RFP	−0.07 (0.06)	0.24	0.11 (0.06)	0.08
AFP	0.07 (0.06)	0.28	0.02 (0.06)	0.78
SOPDF×RFP	−0.01 (0.06)	0.88	−0.13 (0.06)	0.02
SOPDF×AFP	0.17 (0.07)	0.02	0.09 (0.07)	0.20
Age	—	—	0.03 (0.05)	0.56
Relationship status[a]	—	—	−0.13 (0.05)	0.006
Monthly salary	—	—	0.13 (0.05)	0.02

Abbreviations: AFP, authoritarian filial piety; RFP, reciprocal filial piety; SOPDF, sexual orientation-based prejudice and discrimination in family of origin.

[a]Relationship status was dummy coded, 0 = having a same-sex partner, 1 = having no same-sex partner or unsure.

TABLE 5 Conditional effect of SOPDF on depressive symptoms and life satisfaction at different values of reciprocal and authoritarian filial piety.

		Depressive symptoms			Life satisfaction		
		B	SE	*p*	*B*	SE	*p*
Reciprocal filial piety as moderator	Very low (−2 SD)	—	—	—	−0.03	0.17	0.87
	Low (−1 SD)	—	—	—	−0.19	0.11	0.10
	Average	—	—	—	−0.34	0.08	<0.001
	High (+ 1 SD)	—	—	—	−0.50	0.10	<0.001
	Very high (+ 2 SD)	—	—	—	−0.66	0.16	<0.001
Authoritarian filial piety as moderator	Very low (−2 SD)	−0.17	0.14	0.23	—	—	—
	Low (−1 SD)	0.01	0.07	0.91	—	—	—
	Average	0.18	0.04	<0.001	—	—	—
	High (+ 1 SD)	0.35	0.10	<0.001	—	—	—
	Very high (+ 2 SD)	0.53	0.17	0.002	—	—	—

Abbreviation: SOPDF, sexual orientation-based prejudice and discrimination in family of origin.

endorsed higher levels of AFP; the negative effect of SOPDF on life satisfaction was stronger for participants with higher levels of RFP.

Consistent with our first hypothesis, SOPDF had a significant contribution to depressive symptoms and life satisfaction. This finding supports the theorized links between distal stressors and mental health outcomes as proposed by the minority stress theory and adds external validity to the applicability of the theory to a non-Western cultural context. Furthermore, this finding underscores the importance of familial relationships in influencing the health of LGBQ+ individuals (Farr et al., 2019; LaSala, 2007; Newcomb et al., 2019). This study also provides empirical evidence for the detrimental impact of anti-LGBQ+ sentiment in family-of-origin on the psychological health of LGBQ+ individuals in China. In addition, the magnitude of the effect of SOPDF on depressive symptoms was comparable to that of the effect of SOPDF on life satisfaction. That indicates that SOPDF posed a detrimental impact on negative and positive mental health to a similar degree. Given the frequent occurrence of sexual prejudice and discrimination in family of origin (Beijing LGBT Center, 2014; Suen et al., 2022) and its negative impact on

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/famp.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

mental health for LGBQ+ individuals in China, finding ways to reduce SOPDF is crucial and urgent. To locate which variables may reduce or strengthen the contribution of SOPDF to positive and negative mental health, this study included AFP and RFP as moderators.

AFP as a moderator

When examining the role of AFP as a moderator, we observed that high levels of AFP intensified the positive impact of SOPDF on depressive symptoms. This suggests that LGBQ+ individuals endorsing higher levels of AFP may be impacted by SOPDF to a greater extent when considering depressive symptoms as a mental health outcome. AFP is characterized by individuals' adherence to obedient relationships with their parents and a sense of obligation to bring honor to the family (Bedford & Yeh, 2019). Those with elevated levels of AFP tend to value belongingness to their family and society. When faced with anti-LGBQ+ sentiment within their family of origin, these individuals may struggle to assert their personal boundaries, as they have learned to prioritize familial needs over their own to avoid being disowned. Over time, such homonegative messages may get internalized. Hence, the combination of fear of disownment and suppression of autonomy contributes to heightened depressive symptoms resulting from SOPDF (Su et al., 2013; Williamson et al., 2020). As such, the link between SOPDF and depressive symptoms was particularly strong among LGBQ+ individuals who endorsed high levels of AFP.

However, we also found that AFP did not moderate the association between SOPDF and life satisfaction. This suggests that AFP may amplify the impact of SOPDF on psychopathology more so than well-being, as AFP emphasizes individuals' obedience and submission to hierarchical authority and suppression of individuating autonomy, rather than the meaning and quality of the parent–child relationship (Bedford & Yeh, 2019; Sun et al., 2019). Such features of AFP may foster negative traits, such as feelings of worthlessness, helplessness, and frustration (Leung et al., 2010), and cognitive conservatism (Jen et al., 2019; Yeh & Bedford, 2003), which are more closely associated with negative mental health, such as depressive symptoms, than positive mental health outcomes.

RFP as a moderator

Higher levels of RFP exacerbated the negative impact of SOPDF on life satisfaction, suggesting that LGBQ+ individuals with higher levels of RFP experience lower life satisfaction in association with SOPDF. Individuals who endorse high levels of RFP tend to value mutual relatedness with their parents (Bedford & Yeh, 2019). Greater RFP was associated with greater life satisfaction through a sense of autonomy with an emphasis of relational harmony (Sun et al., 2019). While previous studies found that RFP may protect LGBQ+ individuals from internalizing public stigma (Liu et al., 2021b) and that RFP was positively linked with life satisfaction (Chen et al., 2016; Sun et al., 2019), this study suggests a different dynamic when stigma is experienced within one's family. Knowing that their parents adhere to heteronormative values, these individuals may feel disappointed by the lack of affection and validation from their family, and may lose hope in maintaining familial harmony if they resist such biases. This sense of grief and loss may explain the stronger negative link between SOPDF and life satisfaction among individuals with high RFP compared to those with low RFP.

However, RFP did not moderate the association between SOPDF and depressive symptoms. It is plausible that individuals endorsing RFP attribute meaning to their lives through maintaining positive familial relationships. RFP may amplify the impact of SOPDF on positive mental health outcomes, such as life satisfaction, as these outcomes involve individuals' self-actualization and

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/famp.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

aspirations beyond symptoms. RFP is characterized by individuals' voluntary support and care for their parents as a way of appreciating their parents' efforts in nurturing them (Bedford & Yeh, 2019). These characteristics may help children develop positive traits, such as empathy (Yeh & Bedford, 2003), emotional intelligence (Chen et al., 2018), and cognitive flexibility (Jen et al., 2019), which are proximal indicators of positive mental health (Wu et al., 2021).

Limitations and future directions

Although this study has deepened our understanding about the interaction effect between filial piety beliefs and SOPDF on mental health among the Chinese sexual minority population, it has several limitations. First, this study used convenience sampling to collect a LGBQ+ sample that were primarily young and highly educated. A lack of representativeness may limit the generalization of the present findings to LGBQ+ individuals who are older and have lower attainment of education. Second, this study did not assess sexual orientation-based prejudice and discrimination from specific figures in the family of origin, such as parents and siblings. Given that AFP and RFP prescribe children's motivation to return care and respect from parents, it is unclear whether AFP and RFP would play different moderating roles in the effects of sexual orientation-based prejudice and discrimination from parents and sibling on mental health. Third, this study adopted a cross-sectional design to study the concurrent association among variables in the hypothesized moderation model. Given that SOPDF could change across time, future researchers are suggested to conduct longitudinal study or use ecological momentary assessment (e.g., daily diary method) to examine the temporal and causal relationship among the variables. Fourth, this study only focused on the LGBQ+ individuals' perceptions, omitting other family members' subjective experience in contributing to LGBQ+ individuals' mental health. Study designs that incorporate multiple informants of LGBQ+ individuals' family may add complexity to the understanding of LGBQ+ individuals' mental health by considering family dynamics. For instance, using analytic strategies such as actor-partner interdependence modeling, researchers may examine how filial piety beliefs of both LGBQ+ individuals and their parents contribute to parent–child conflicts on LGBQ+-related issues and wellbeing.

It is important to note that this study did not differentiate LGBQ+ individuals based on their family type, specifically whether they come from heterosexual or sexual minority families. Existing research conducted in Western contexts has indicated that children from sexual minority families tend to exhibit lower or comparable levels of psychological distress compared to their counterparts from heterosexual families (Green et al., 2019; Patterson, 2022). Therefore, it remains a question whether the levels of SOPDF, filial piety beliefs, mental health outcomes, and their interrelationships would vary depending on their family type among Chinese LGBQ+ individuals.

Considering filial piety as a value shaped by one's culture and environment, researchers may test the generalizability of our observed findings across cultures to extend the impact of this study. People socialized in individualistic societies tend to prioritize autonomy and independence over duties and obligations (Brewer & Chen, 2007), different from people's emphasis on relationships and interdependence in collectivistic cultures where our sample was situated. As a result of the different cultural norms, AFP and RFP may function differently. For instance, it is plausible that filial piety beliefs may exert a weaker or nonsignificant moderating role in the impact of SOPDF on mental health outcomes for people living in individualistic societies. Furthermore, research has indicated a positive link between religiosity, especially at extreme ends, and filial norms and behaviors among samples in Western nations (Gans et al., 2009), suggesting that religiosity may influence the expression of filial piety. Researchers could explore how religiosity might similarly moderate the link between SOPDF and mental health among LGBQ+ people. These findings would add complexity to clinicians' understanding of

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/famp.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

the lived experiences of LGBQ+ clients by considering the interplay between religiosity and filial piety.

Implications

Despite these limitations, this study has significant implications for practice, education/training, advocacy, and theory. In terms of practice, mental health professionals may need to assess clients' SOPDF, AFP, and RFP, and pay attention to clients who endorse high levels of AFP and experience high levels of SOPDF when working with Chinese LGBQ+ clients because these clients may be more prone to experience depressive symptoms. Therapists can support their LGBQ+ clients to creatively identity what healthy boundaries may "look like" in their unique familial context. Understanding clients' endorsement of different types of filial piety beliefs is valuable for therapists as it helps them grasp the dynamics of the therapeutic collaboration and support clients in cultivating assertiveness skills and other strategies to address SOPDF. For instance, when working with LGBQ+ clients who exhibit high levels of AFP, therapists can be attentive to their expectations for the therapist to take a more directive role in counseling. On the other hand, when working with LGBQ+ clients embracing RFP, therapists can explore their inclination to reciprocate the care provided by the therapist. By raising awareness of these possible dynamics during therapy, clients can gain insight into the advantages and drawbacks of their interpersonal style in relation to their filial piety beliefs. In the case of therapists endorsing family therapy to assist LGBQ+ clients in navigating SOPDF, it becomes important to help the clients' family members, especially parents, grieve the idealized heterosexual image they may hold of their child. Therapists can also facilitate understanding of the diverse lived experiences of non-heterosexual individuals to promote a reduction in prejudice and discrimination within the family (LaSala, 2000). Additionally, these therapists should consider how LGBQ+ clients' filial piety beliefs may amplify the impact of familial sexual stigma on their mental health when providing LGBQ+ affirmative family therapy.

In terms of education and training, LGBQ+ non-profit organizations, such as the Beijing LGBT Center, could provide lectures, workshops, and training programs for Chinese sexual minority individuals to learn how to understand and deal with prejudice and discrimination from family. In addition, these organizations could also support family members of LGBQ+ individuals to gain awareness about the intensifying effect of both types of filial piety on the detrimental impact of familial anti-LGBQ+ sentiment. While some parents may be concerned about coping and well-being of their children (e.g., in response to the larger heteronormative culture), others may be worried about losing face as a family (Huang & Brouwer, 2018; Ren et al., 2019). Family members of LGBQ+ individuals may benefit from recognizing internal forces that drive behaviors perceived as anti-LGBQ+ by their children. Furthermore, it is imperative to enact LGBQ+-inclusive policies, such as the legal recognition and protection of same-sex relationships and the integration of gender and sexuality diversity education within schools and families. These initiatives are crucial for fostering a safe and supportive environment for LGBQ+ individuals within both familial and societal spheres. In terms of theory, the moderating roles of AFP and RFP in the relation between SOPDF and psychological health uncovered in the present study informed us that contextual variables should be taken into account when using the minority stress theory to understand LGBQ+ individuals' psychological health.

FUNDING INFORMATION

This study was supported by the research fund awarded from the Key Research Base of Humanities and Social Sciences in Universities of Jiangxi Province to Fangsong Liu (JD23060) and the Seed Fund for Basic Research awareded from the University of Hong Kong to Eddie Chong (202107185039).

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/famp.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

DATA AVAILABILITY STATEMENT

The data that support the findings of this study are available from the corresponding author upon reasonable request.

ORCID

Fangsong Liu https://orcid.org/0000-0002-2615-3721
Eddie S. K. Chong https://orcid.org/0000-0002-9601-4372

REFERENCES

Anderson, J. C., & Gerbing, D. W. (1988). Structural equation modeling in practice: A review and recommended two-step approach. *Psychological Bulletin*, *103*(3), 411–423. https://doi.org/10.1037/0033-2909.103.3.411

Bariola, E., Lyons, A., & Lucke, J. (2017). Flourishing among sexual minority individuals: Application of the dual continuum model of mental health in a sample of lesbians and gay men. *Psychology of Sexual Orientation and Gender Diversity*, *4*(1), 43–53. https://doi.org/10.1037/sgd0000210

Bedford, O., & Yeh, K. H. (2019). The history and the future of the psychology of filial piety: Chinese norms to contextualized personality construct. *Frontiers in Psychology*, *10*, 100. https://doi.org/10.3389/fpsyg.2019.00100

Bedford, O., & Yeh, K. H. (2021). Evolution of the conceptualization of filial piety in the global context: From skin to skeleton. *Frontiers in Psychology*, *12*, 570547. https://doi.org/10.3389/fpsyg.2021.570547

Beijing LGBT Center. (2014). *Chinese LGBT mental health survey report*. Beijing, China.

Brewer, M. B., & Chen, Y. R. (2007). Where (who) are collectives in collectivism? Toward conceptual clarification of individualism and collectivism. *Psychological Review*, *114*(1), 133–151. https://doi.org/10.1037/0033-295x.114.1.133

Chan, R. C. H., Leung, J. S. Y., & Wong, D. C. K. (2022). Experiences, motivations, and impacts of sexual orientation change efforts: Effects on sexual identity distress and mental health among sexual minorities. *Sexuality Research & Social Policy*, *19*(4), 1534–1548. https://doi.org/10.1007/s13178-021-00669-5

Chen, S., Chiu, H., Xu, B., Ma, Y., Jin, T., Wu, M., & Conwell, Y. (2010). Reliability and validity of the PHQ-9 for screening late-life depression in Chinese primary care. *International Journal of Geriatric Psychiatry*, *25*(11), 1127–1133. https://doi.org/10.1002/gps.2442

Chen, W. W., Wu, C. W., & Yeh, K. H. (2016). How parenting and filial piety influence happiness, parent–child relationships and quality of family life in Taiwanese adult children. *Journal of Family Studies*, *22*(1), 80–96. https://doi.org/10.1080/13229400.2015.1027154

Chen, W. W., Yan, J. J., & Chen, C. C. (2018). Lesson of emotions in the family: The role of emotional intelligence in the relation between filial piety and life satisfaction among Taiwanese college students. *Asian Journal of Social Psychology*, *21*(1–2), 74–82. https://doi.org/10.1146/annurev.clinpsy.1.102803.144154

Choi, S. Y., & Luo, M. (2016). Performative family: Homosexuality, marriage and intergenerational dynamics in China. *The British Journal of Sociology*, *67*(2), 260–280. https://doi.org/10.1111/1468-4446.12196

Cummins, R. A. (2013). Measuring happiness and subjective well-being. In S. David, I. Boniwell, & A. Conley Ayers (Eds.), *The Oxford handbook of happiness* (pp. 185–200). Oxford University Press.

Davis, K. C., & Anderson, J. L. (2021). The role of perceived parental acceptance-rejection on personality psychopathology in sexual orientation minority young adults. *Journal of Child and Family Studies*, *30*, 276–291. https://doi.org/10.1007/s10826-020-01858-3

DeChants, J. P., Shelton, J., Anyon, Y., & Bender, K. (2022). "It kinda breaks my heart": LGBTQ young adults' responses to family rejection. *Family Relations*, *71*(3), 968–986. https://doi.org/10.1111/fare.12638

Diener, E. D., Emmons, R. A., Larsen, R. J., & Griffin, S. (1985). The satisfaction with life scale. *Journal of Personality Assessment*, *49*(1), 71–75. https://doi.org/10.1207/s15327752jpa4901_13

Farr, R. H., Bruun, S. T., & Patterson, C. J. (2019). Longitudinal associations between coparenting and child adjustment among lesbian, gay, and heterosexual adoptive parent families. *Developmental Psychology*, *55*(12), 2547–2560. https://doi.org/10.1037/dev0000828

Friedman, M. S., Marshal, M. P., Guadamuz, T. E., Wei, C., Wong, C. F., Saewyc, E. M., & Stall, R. (2011). A meta-analysis of disparities in childhood sexual abuse, parental physical abuse, and peer victimization among sexual minority and sexual nonminority individuals. *American Journal of Public Health*, *101*(8), 1481–1494. https://doi.org/10.2105/AJPH.2009.190009

Gans, D., Silverstein, M., & Lowenstein, A. (2009). Do religious children care more and provide more care for older parents? A study of filial norms and behaviors across five nations. *Journal of Comparative Family Studies*, *40*(2), 187–201. https://doi.org/10.3138/jcfs.40.2.187

Green, R. J., Rubio, R. J., Rothblum, E. D., Bergman, K., & Katuzny, K. (2019). Gay fathers by surrogacy. Prejudice, parenting, and well- being of female and male children. *Psychology of Sexual Orientation and Gender Diversity*, *6*(3), 269–283. https://doi.org/10.1037/sgd0000325

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/famp.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

Hu, L., & Bentler, P. M. (1999). Cut-off criteria for fit indexes in covariance structure analysis: Conventional criteria versus new alternatives. *Structural Equation Modeling*, *6*(1), 1–55. https://doi.org/10.1080/10705519909540118

Huang, S., & Brouwer, D. C. (2018). Coming out, coming home, coming with: Models of queer sexuality in contemporary China. *Journal of International and Intercultural Communication*, *11*(2), 97–116. https://doi.org/10.1080/17513057.2017.1414867

Jen, C. H., Chen, W. W., & Wu, C. W. (2019). Flexible mindset in the family: Filial piety, cognitive flexibility, and general mental health. *Journal of Social and Personal Relationships*, *36*(6), 1715–1730. https://doi.org/10.1177/0265407518770912

Kam, L. Y. L. (2007). Noras on the road: Family and marriage of lesbian women in Shanghai. *Journal of Lesbian Studies*, *10*(3–4), 87–103. https://doi.org/10.1300/J155v10n03_06

Keyes, C. L. (2005). Mental illness and/or mental health? Investigat- ing axioms of the complete state model of health. *Journal of Consulting and Clinical Psychology*, *73*(3), 539–548. https://doi.org/10.1037/0022-006x.73.3.539

Kline, R. B. (2005). *Principle and practice of structural equation modeling* (2nd ed.). Guilford Press Publications.

Kroenke, K., Spitzer, R. L., & Williams, J. B. W. (2001). The PHQ-9: Validity of a brief depression severity measure. *Journal of General Internal Medicine*, *16*(9), 606–613. https://doi.org/10.1046/j.1525-1497.2001.016009606.x

LaSala, M. C. (2000). Lesbians, gay men, and their parents: Family therapy for the coming-out crisis. *Family Process*, *39*(1), 67–81. https://doi.org/10.1111/j.1545-5300.2000.39108.x

LaSala, M. C. (2007). Parental influence, gay youths, and safer sex. *Health & Social Work*, *32*(1), 49–55. https://doi.org/10.1093/hsw/32.1.49

Leung, A. N. M., Wong, S. S. F., Wong, I. W. Y., & McBride-Chang, C. (2010). Filial piety and psychosocial adjustment in Hong Kong Chinese early adolescents. *The Journal of Early Adolescence*, *30*(5), 651–667. https://doi.org/10.1177/0272431609341046

Li, Y. H., & Zheng, H. X. (2013). Public attitudes toward homosexuality and its correlated factors. *Journal of South China Normal University (Social Science Edition)*, *6*(6), 31–36.

Lindert, J., Bain, P. A., Kubzansky, L. D., & Stein, C. (2015). Well-being measurement and the WHO health policy health 2010: Systematic review of measurement scales. *The European Journal of Public Health*, *25*(4), 731–740. https://doi.org/10.1093/eurpub/cku193

Liu, F., & Chong, E. S. (2023). On the link between parental support and life satisfaction: General and LGBQ+ specific self-concepts as mediators, and parent-oriented interdependent self-construal as a moderator. *Journal of Social and Personal Relationships*, *40*(10), 3392–3413. https://doi.org/10.1177/02654075231174187

Liu, F., Chong, E. S., & Chui, H. (2022). Examining interpersonal mediators and moderators in the link between outness to family and depressive symptoms among Chinese sexual minorities. *Current Psychology*, *1-13*, 25717–25729. https://doi.org/10.1007/s12144-022-03662-2

Liu, F., Chui, H., & Chung, M. C. (2021a). Reciprocal/authoritarian filial piety and mental well-being in the Chinese LGB population: The roles of LGB-specific and general interpersonal factors. *Archives of Sexual Behavior*, *51*, 3513–3527. https://doi.org/10.1007/s10508-021-02130-5

Liu, F., Chui, H., & Chung, M. C. (2021b). The moderating effect of filial piety on the relationship between perceived public stigma and internalized homophobia: A national survey of the Chinese LGB population. *Sexuality Research & Social Policy*, *18*(1), 160–169. https://doi.org/10.1007/s13178-020-00446-w

Mann, S. L. (2011). *Gender and sexuality in modern Chinese history* (Vol. 9). Cambridge University Press. https://doi.org/10.1017/CBO9781139013307

Marsh, H. W., Wen, Z., & Hau, K. T. (2004). Structural equation models of latent interactions: Evaluation of alternative estimation strategies and indicator construction. *Psychological Methods*, *9*(3), 275–300. https://doi.org/10.1037/1082-989x.9.3.275

Meyer, D., Cohn, A., Robinson, B., & Zubatsky, M. (2022). Differences in life satisfaction and emotions by romantic coupling and sexual orientation. *Journal of Family Issues*, *44*(10), 2665–2683. https://doi.org/10.1177/0192513x221106737

Meyer, I. H. (2003). Prejudice, social stress, and mental health in lesbian, gay, and bisexual populations: Conceptual issues and research evidence. *Psychological Bulletin*, *129*(5), 674–697. https://doi.org/10.1037/0033-2909.129.5.674

Michaels, C., Choi, N. Y., Adams, E. M., & Hitter, T. L. (2019). Testing a new model of sexual minority stress to assess the roles of meaning in life and internalized heterosexism on stress-related growth and life satisfaction. *Psychology of Sexual Orientation and Gender Diversity*, *6*(2), 204–216. https://doi.org/10.1037/sgd0000320

Newcomb, M. E., LaSala, M. C., Bouris, A., Mustanski, B., Prado, G., Schrager, S. M., & Huebner, D. M. (2019). The influence of families on LGBTQ youth health: A call to action for innovation in research and intervention development. *LGBT Health*, *6*(4), 139–145. https://doi.org/10.1089/lgbt.2018.0157

Parra, L. A., Bell, T. S., Benibgui, M., Helm, J. L., & Hastings, P. D. (2018). The buffering effect of peer support on the links between family rejection and psychosocial adjustment in LGB emerging adults. *Journal of Social and Personal Relationships*, *35*(6), 854–871. https://doi.org/10.1177/0265407517699713

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/fam.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

Patterson, C. J. (2022). Parental sexual orientation, parental gender identity, and the development of children. In *Advances in child development and behavior* (Vol. *63*, pp. 71–102). Elsevier.

Pew Research Center. (2013). *A survey of LGBT Americans*. http://www.pewsocialtrends.org/2013/06/13/a-survey-of-lgbt-americans/

Puckett, J. A., Woodward, E. N., Mereish, E. H., & Pantalone, D. W. (2015). Parental rejection following sexual orientation disclosure: Impact on internalized homophobia, social support, and mental health. *LGBT Health*, *2*(3), 265–269. https://doi.org/10.1089/lgbt.2013.0024

Ren, Z., Howe, C. Q., & Zhang, W. (2019). Maintaining "mianzi" and "lizi": Understanding the reasons for formality marriages between gay men and lesbians in China. *Transcultural Psychiatry*, *56*(1), 213–232. https://doi.org/10.1177/1363461518799517

Rosario, M., Reisner, S. L., Corliss, H. L., Wypij, D., Frazier, A. L., & Austin, S. B. (2014). Disparities in depressive distress by sexual orientation in emerging adults: The roles of attachment and stress paradigms. *Archives of Sexual Behavior*, *43*(5), 901–916. https://doi.org/10.1007/s10508-013-0129-6

Russell, D. W., Kahn, J. H., Spoth, R., & Altmaier, E. M. (1998). Analyzing data from experimental stud- ies: A latent variable structural equation modeling approach. *Journal of Counseling Psychology*, *45*(1), 18–29. https://doi.org/10.1037/0022-0167.45.1.18

Shao, J., Chang, E. S., & Chen, C. (2018). The relative importance of parent–child dynamics and minority stress on the psychological adjustment of LGBs in China. *Journal of Counseling Psychology*, *65*(5), 598–604. https://doi.org/10.1037/cou0000281

Su, J. C., Lee, R. M., & Oishi, S. (2013). The role of culture and self-construal in the link between expressive suppression and depressive symptoms. *Journal of Cross-Cultural Psychology*, *44*(2), 316–331. https://doi.org/10.1177/0022022112443413

Suen, Y. T., Wong, E. M. Y., & Chan, R. C. (2022). Chinese lesbian and gay adults' self-reported experiences of negative treatment and violence from family of origin: Evidence from a larger-scale study in China. *Journal of Family Issues*, *44*, 1880–1903. https://doi.org/10.1177/0192513x211064874

Sun, P., Fan, X., Sun, Y., Jiang, H., & Wang, L. (2019). Relations between dual filial piety and life satisfaction: The mediating roles of individuating autonomy and relating autonomy. *Frontiers in Psychology*, *10*, 2549. https://doi.org/10.3389/fpsyg.2019.02549

Szymanski, D. M. (2006). Does internalized heterosexism moderate the link between heterosexist events and lesbians' psychological distress? *Sex Roles*, *54*(3–4), 227–234. https://doi.org/10.1007/s11199-006-9340-4

Szymanski, D. M. (2009). Examining potential moderators of the link between heterosexist events and gay and bisexual men's psychological distress. *Journal of Counseling Psychology*, *56*(1), 142–151. https://doi.org/10.1037/0022-0167.56.1.142

Tang, J., & Pan, J. (2018). Time trends and group differences in public tolerance toward homosexual behavior in China. *Chinese Journal of Clinical Psychology*, *26*(6), 1218–1221. https://doi.org/10.16128/j.cnki.1005-3611.2018.06.037

Williamson, T. J., Ostroff, J. S., Haque, N., Martin, C. M., Hamann, H. A., Banerjee, S. C., & Shen, M. J. (2020). Dispositional shame and guilt as predictors of depressive symptoms and anxiety among adults with lung cancer: The mediational role of internalized stigma. *Stigma and Health*, *5*(4), 425–433. https://doi.org/10.1037/sah0000214

Willoughby, B. L., Doty, N. D., & Malik, N. M. (2010). Victimization, family rejection, and outcomes of gay, lesbian, and bisexual young people: The role of negative GLB identity. *Journal of GLBT Family Studies*, *6*(4), 403–424. https://doi.org/10.1080/1550428x.2010.511085

World Health Organization. (2017). *Depression and other common mental disorders: Global health estimates*. World Health Organization. https://iris.who.int/bitstream/handle/10665/254610/WHO-MSD-MER-2017.2-eng.pdf

Wu, C. H., & Yao, G. (2006). Analysis of factorial invariance across gender in the Taiwan version of the satisfaction with life scale. *Personality and Individual Differences*, *40*(6), 1259–1268. https://doi.org/10.1016/j.paid.2005.11.012

Wu, C. W., Chen, W. W., & Jen, C. H. (2021). Emotional intelligence and cognitive flexibility in the relationship between parenting and subjective well-being. *Journal of Adult Development*, *28*(2), 106–115. https://doi.org/10.1007/s10804-020-09357-x

Yeh, K. H., & Bedford, O. (2003). A test of the dual filial piety model. *Asian Journal of Social Psychology*, *6*(3), 215–228. https://doi.org/10.1046/j.1467-839x.2003.00122.x

How to cite this article: Liu, F., & Chong, E. S. K. (2024). Effects of sexual orientation-based prejudice and discrimination in family of origin on depressive symptoms and life satisfaction in a Chinese sample: Reciprocal and authoritarian filial piety as moderators. *Family Process*, *63*(4), 2416–2433. https://doi.org/10.1111/famp.13012

15455300, 2024, 4, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/famp.13012, Wiley Online Library on [17/12/2025]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License